



WNHS PROCEDURE

Referral to Women and Newborn Health Service (WNHS) Mental Health Service

Scope (Staff)	Clinical and non-clinical staff working within WNHS Mental Health Service (MHS)
Scope (Area)	WNHS MHS – Consultation and Consumer Liaison (previously known as Department of Psychological Medicine)
Summary	WNHS MHS provides comprehensive care and support to women and/or their babies recognising the need for both physical and mental health care. The purpose of this procedure is to ensure all referrals to the WNHS MHS are received, and responded to in a coordinated, timely and clinically appropriate manner consistent with the Mental Health Act 2014 and Operational Directive 0530/14 WA Health Specialist Outpatient Services Access Policy. This procedure has undergone three yearly review with the following changes: • Reflect name change from Department of Psychological Medicine to WNHS MHS: Outpatient and Consultation Liaison • SAC 1 Recommendation: CIMS371894/ DATIX ID 470095
Approval	Mental Health Clinical Practice Outcome Committee
Sponsor	Head of Psychiatry, WNHS MHS
Contact	Head of Psychiatry – WNHS MHS
Version	2.0
Review Date	13 May 2027
NSQHS Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2) Comprehensive Care (Std 5) Communicating for Safety (Std 6) Recognising and Responding to Acute Deterioration (Std 8)
Status	FINAL

1. Aim

The purpose of this procedure is to ensure all referrals to the WNHS MHS are received, and responded to in a coordinated, timely and clinically appropriate manner consistent with the [Mental Health Act 2014](#) and Operational Directive 0530/14 [WA Health Specialist Outpatient Services Access Policy](#).

The WNHS MHS aspires to ensure the rights of a woman and partner/support person are met according to the [Charter of Mental Health Care Principles](#) (*Mental Health Act 2014*).

As per the Department of Health [MP 0181/24 Safety Planning for Mental Health Consumers Policy](#) this procedure and its underpinning guidelines aims to promote care that is:

- Recovery-oriented
- Person-centred
- Trauma-informed
- Culturally competent
- Developmentally appropriate.

2. Risk

Non-compliance with this policy will breach legislative and mandatory policy requirements. Further, non-compliance may result in an impact on patient safety, staff safety, quality of patient care and reputational image.

3. Inpatient Referral

- 3.1 Wherever possible, inpatients should provide consent prior to referral being made to WNHS MHS.
- 3.2 Patients from non-English speaking background with a hearing impairment shall be offered the assistance of a language or Auslan interpreter or telephone interpreter service. See [WNHS Language Services](#).
- 3.3 For people of Aboriginal descent, to the extent that is practicable and appropriate, with the patient's consent, initial assessments should be conducted in collaboration with an Aboriginal mental health worker or significant member of the person's community. See [NMHS Mandatory Training Policy](#).
- 3.4 All patients and their partner/support person (with patient consent) are included in decisions about their treatment and follow up.
- 3.5 Patients are informed that it is best practice to communicate with other relevant health professionals involved in their care, only as it is relevant and important to their care.
- 3.6 Inpatient referrals are made via the E-Referral system and received via WNHS MHS triage assessment.
- 3.7 Referrals may also occur directly to and be triaged by clinicians working within MDT teams and clinics in the hospital.
- 3.8 The E-Referral form must be completed as thoroughly as possible, including the urgency of referral.



- 3.9** For urgent referrals outside of work hours, the WNHS MHS on-call Medical Officer should be paged via the hospital switch board on 91.
- 3.91** In the event of the referred patient declining an assessment, WNHS MHS will provide feedback to the referrer and where there is low risk of harm to self or other. As a minimum, information on contacts for crisis Mental Health Emergency Response Line (MHERL) 1300 555 788 and Life Line 13 11 14 will be provided.
- 3.92** If the patient appears to have significant risk factors for harm to self or other, WNHS MHS staff will endeavour to engage the patient in assessment and safety planning. Consideration is to be given to the appropriateness of the application of the *Mental Health Act 2014* to ensure immediate patient safety.

4. Inpatient Triage

- 4.1** Triage Officer: Triaging duty is undertaken by the Clinical Nurse Specialist Mental Health Liaison (CNS MHL) in the first instance or by Duty Medical or Duty Psychology.
- 4.2** All referrals are coordinated through the Triage Officer and presented for allocation at the daily team meeting.
- 4.3** Referrals must ensure sufficient patient information is provided in the referral for the Triage Officer to assess and allocate. If not, the referrer or patient will be contacted by the Triage Officer for further information.
- 4.4** Routine referrals received after midday will be allocated for clinician follow up by the Triage Officer the following morning.
- 4.5** The Triage Officer can be contacted on 81521 during operating hours 8.30 to 4.30pm or via email kemh.psychmedtriage@health.wa.gov.au Monday to Friday.
- 4.6** If the referral does not meet eligibility requirements for acceptance to clinical services in WNHS MHS, the triaging clinician will provide information to the patient or referrer as appropriate about relevant services, document this in the patient's medical file and inform the referrer.
- 4.7** WNHS MHS will respond as quickly as possible to psychiatric emergencies (e.g., acute risk to self or others, acute disturbance).
- 4.8** Urgent referrals (e.g., high level of distress, high risk and time sensitive) are seen on the same day or as soon as possible. Efforts are made for same day review if patients are due to be discharged.
- 4.9** Urgent referrals received after hours or on the weekend are considered by the on-call WNHS MHS medical or psychiatry staff.
- 4.91** An initial assessment (including risk assessment) is conducted for all new inpatients.
- 4.92** Assessment information is documented in the Medical Digital Record.
- 4.93** Risk assessment will form part of the standard initial and ongoing assessments of all inpatients seen by WNHS MHS. This is documented in the [Mental Health Risk Assessment and Management Plan SMHMR905](#)
- 4.94.1** Inpatients referred to WNHS MHS who are discharged prior to Initial Assessment are allocated to the outpatient process for follow up.

5 Outpatient Referral

- 5.1** WNHS MHS accepts outpatient referrals from
- General Hospital KEMH Ward
 - Hospital Outpatient Clinics
 - Visiting Midwifery Service



- Community Mental Health Nurses
- Family members (with patient consent)
- Self-referral
- Other relevant external agencies
- General Practitioner

- 5.1** Only patients of WNHS or mothers of babies who have been hospitalised in Special Care Nursery (SCN) or Ward 3B Perth Children's Hospital (PCH) are eligible for WNHS MHS.
- 5.2** Patients can be seen whilst they are ongoing patients of the Hospital Clinics, and up to 12 months post-birth for perinatal women as clinically appropriate, for mental health issues.
- 5.3** External referrals should be made on the WNHS MHS [Referral Form \(MR208.13\)](#) or via letter. Internal to WNHS MHS referrals should be made via the E-referral form. Patient consent should be noted.

6 Outpatient Triage

- 6.1** The referral is received by the Triage Officer, discussed at the daily multidisciplinary team meeting, or with a relevant senior clinician, and allocated to Psychiatry or Clinical Psychology for an **Initial Assessment (IA)** according to clinical need.
- 6.2** Every attempt is made to contact the patient firstly by phone, then SMS and finally by letter with WNHS MHS information brochure.
- 6.3** If the patient does not respond to the initial assessment letter within four weeks the Triage Officer will document in the Medical Digital Record, inform the referrer where appropriate and close the referral.
- 6.4 Appointment Scheduling**
Scheduled Initial Assessments are Documented in accordance with the [Statewide Standardised Clinical Documentation \(SSCD\)](#). Risk assessment will form part of the standard initial and on-going assessments of all outpatients seen by WNHS MHS. This is documented in the [Mental Health Risk Assessment and Management Plan SMHMR905](#)
- 6.5 Urgent Triage Scheduled Assessment IA:** High risk or urgent referrals are given priority and emergency assessments are undertaken.
- 6.6 Scheduled Assessment IA:** Routine Initial Assessment appointment availability can vary according to patient needs and staffing resources.
- 6.7 Rescheduling Scheduled Assessment IA:** When rescheduling an initial assessment, the administration team must provide the patients paperwork to the multidisciplinary team morning meeting for the duty clinician (either Psychology or Psychiatry) to assess the risk prior to the patient being contacted for rescheduling. The reappointment of the initial assessment is a decision made in the multidisciplinary team meeting.
- 6.8 High risk:** the patient is assessed as high risk the duty clinician will telephone/contact the patient after the multidisciplinary meeting to assess the clinical risk and determine the level of follow-up required.
- 6.9 Low risk:** if the patient is assessed as low risk, the paperwork will be returned to the administration team. Administrative staff will be advised that the patient can be allocated with the initial clinician within 10 days. All actions will be documented in the patient's notes as an outpatient entry.
- 6.91 Rescheduling follow-up appointments** (unplanned or sick leave). When a follow-up appointment is required to be rescheduled every effort should be made by the clinician to highlight any patients of concern that require follow up by the duty team on the day. The



treating clinician can indicate a patient of concern to the administration team by emailing the admin shared mailbox and/ or by calling 6458 1521 and leaving a voice mail message. If the clinician is expected to return within the next 2 to 3 working days and has clearly indicated no patient of concern in their booked appointments, the administration team will reschedule all appointments to the next available.

- 6.92** If the treating clinician is not able to indicate a patient of concern and unlikely to return the next day the administration officer will provide a list of all patients who need rescheduling and have not been reviewed by the clinician who is absent, to the duty or senior clinician for consideration.
- 6.93** It is the responsibility of the duty clinician and/or senior clinician to review the Medical Digital Records for booked appointments to assess for deemed risk and need for urgent reviews in the clinician's absence.
- 6.94 High risk:** If the patient is assessed as high risk the duty clinician will telephone/contact the patient after the multidisciplinary meeting to assess the clinical risk and determine the level of follow-up required for the consumer.
- 6.95 Low risk:** If the patient is assessed as low risk, the paperwork will be returned to the administration team. Administrative staff will be advised that the patient can be allocated to the next available appointment with the treating clinician. All actions will be documented in the patient's medical notes as an outpatient entry.

Note: If administration staff has any concerns or experience any issues when speaking to the patient, the appointment should be booked and the patient will be informed that the duty clinician will call them back to discuss their concerns.

Client Did Not Attend (DNA) Appointment Scheduled Assessment IA

A patient may fail to attend (DNA) due to deterioration in mental health. In line with the [WNHS Outpatient Referrals \(Management\) Policy](#) and the [Department of Health Specialist Outpatient Services Access Policy](#) the WNHS MHS will exercise discretion on a case-by-case basis to avoid disadvantaging patients in the case of genuine hardship, misunderstanding and other unavoidable circumstances.

Urgent Triage Scheduled Assessment IA

Patients who fail to attend (DNA) an urgent Triage scheduled initial assessment will have a risk assessment completed by the Triage Officer and based on the outcome of the risk evaluation, may need to be assertively followed up within 24 hours by the clinician. This review should be documented in the patient's medical digital notes as an outpatient entry and on Psychiatric Services Online Information System (WebPsolis). Clinical staff will telephone the patient to discuss the DNA and determine the reasons for non-attendance. The clinician will assess the level of follow-up required for the consumer as a matter of priority.

Scheduled Assessment IA: Routine Initial Assessment

If a client DNAs their initial assessment and does not give any reason, then we do not offer a new appointment. For booked appointments the patient receives a confirmation text message and 48 hrs before the appointment an automated reminder message is sent to the patient.

If no risks are identified the clinician will clearly document in the patient's digital medical record and advise clerical staff to send a closing letter to the patient with cc to the referrer.



Did Not Attend (DNA) Scheduled Appointment

If a patient fails to attend (DNA) a follow-up appointment the clinician will call the patient to determine the reasons for the non-attendance. The clinician will assess the level of follow-up required for the consumer as a matter of priority and organise for an assessment. The clinician/admin will organise a new appointment date and time based on clinical needs. The patient will be advised via letter/SMS or phone call as appropriate. If the patient fails to attend the rescheduled appointment the patient will be discharged from the outpatient appointment list and the referrer will be notified. This will be clearly documented in the patient's medical digital record.

Note: If the clinician is concerned about patient's risk, the clinician/admin should ring the patient or a family member and notify the referrer and/or local Acute Treatment Team. The matter should be referred to the multidisciplinary team meeting for discussion. All actions should be documented in the patient's medical digital record.

7. Document Control

Version	Amendment(s)	Published Date
1.0	Published version	09/06/2021
2.0	Three yearly review	13/05/2024

Related Policies, Guidelines, Procedures and Forms

[Women's and Perinatal Mental Health Referral and Management](#)

Operational Directive 0530/14 [WA Health Specialist Outpatient Services Access Policy](#).

[WNHS Language Services Procedure](#)

[NMHS Mandatory Training Policy](#)

[Mental Health Risk Assessment and Management Plan SMHMR905](#)

[WNHS MHS Referral Form Perinatal](#)

[WNHS Outpatient Referrals \(Management\) Policy](#)

[Statewide Standardised Clinical Documentation \(SSCD\)](#)

Definitions

Term	Definition
Multidisciplinary Team (MDT)	A group of health professionals who are members of different disciplines (psychiatrist, psychologist, nurse) each providing specific services to the patient.



Patient of Concern	A person that a clinician is particularly concerned about, as defined by the treating clinician.
Risk Assessment	A gathering of information and analysis of the potential outcomes of identified behaviours. Identifying specific risk factors or relevance to an individual, and the context in which they may occur. This process requires linking historical information to current circumstances, to anticipate possible future change.
Triage Officer	Triaging duty is undertaken by the Clinical Nurse Specialist Mental Health Liaison (CNS MHL) in the first instance or by Duty Medical or Duty Psychology.
<i>Types of Appointments</i>	
Urgent Triage Scheduled Assessment IA (Psychology, Psychiatry)	Booked by Clinical Nurse Specialist during triage assessment.
Scheduled Assessment IA (Initial Assessment)	Psychology or Psychiatry (routine / non-urgent)
Client Appointment	Follow up psychology
Client appointment New Out Patient (OP) Psychology.	Prior completion of Scheduled Assessment by Psychiatry
Medical Appointment	Follow- up psychiatry appointment or Medical Review Assessment

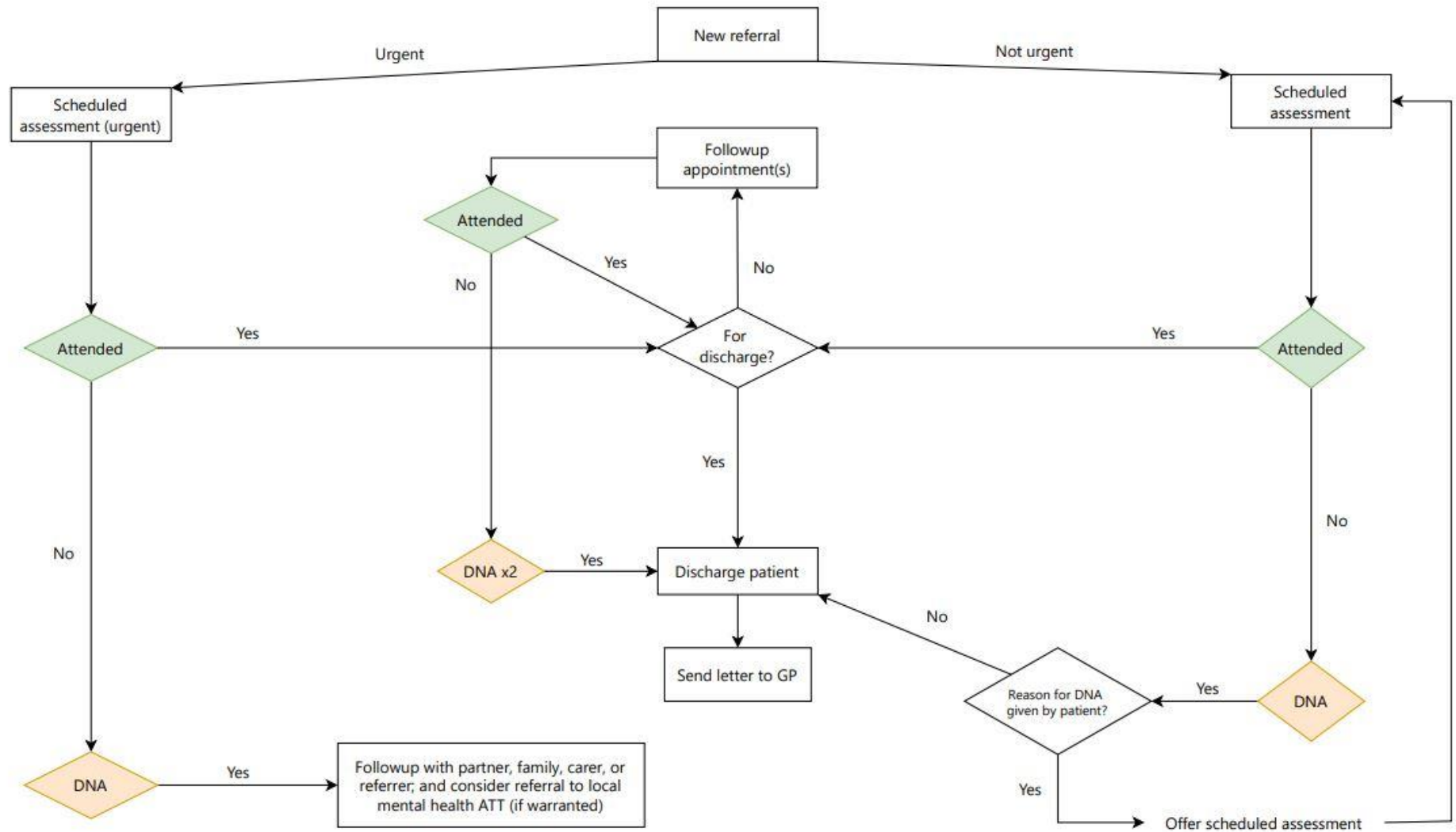
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Appendix 1: WNHS MHS Referral Procedure for New Referrals, Scheduled Assessments, and Follow Up Appointments



Key

ATT = Assessment and treatment team
DNA = Did not attend

Appendix 2: WNHS MHS Referral Procedure when Clinician is not available for Scheduled Assessment

